

Institutional Review Board (IRB) Clinical Protocol

Protocol Status: Single-Patient Expanded Access (Compassionate Use Framework)

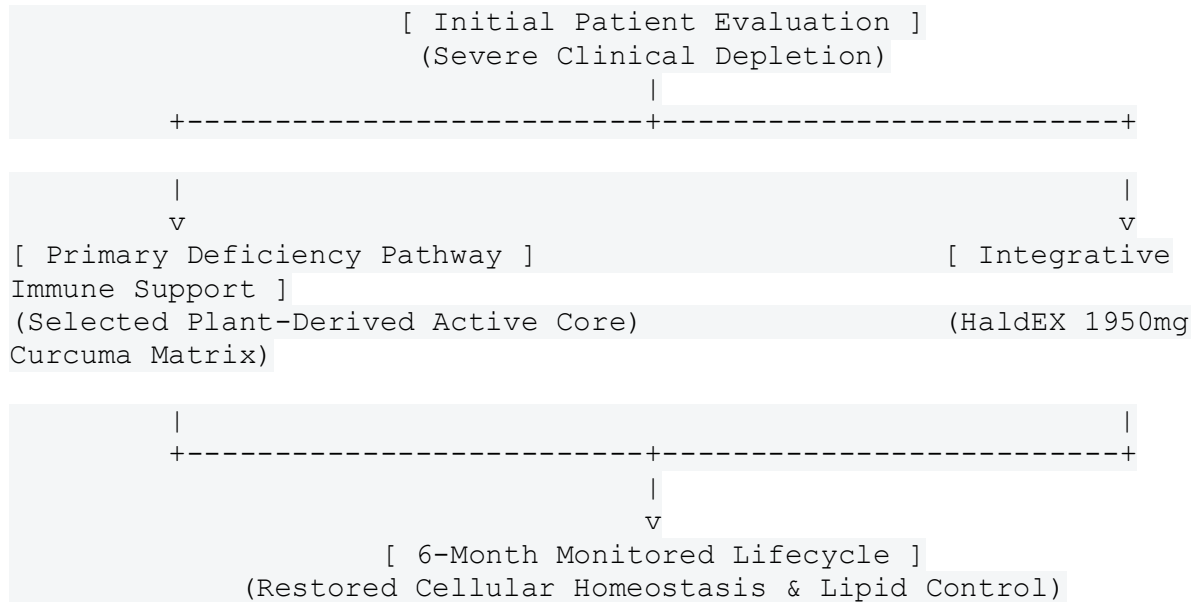
Authorized Phase Duration: 6 Months (Consecutive)

Target Physiological Endpoint: Reversal of Nutrient Deficiency & Resolution of Cellular Oxidative Stress

Financial Status: Voluntary Enrollment; 100% Insurer Funded

I. Rationale & Synergistic Mechanisms

This clinical protocol establishes a managed, 6-month single-patient expanded access framework utilizing an unadulterated, plant-derived molecular approach to address profound systemic nutrient deficiencies and protect cellular integrity from chronic secondary metabolic degradation.



1. **Targeted Deficiency Reversal:** Patients suffering from advanced metabolic depletion, cachexia, or gastrointestinal malabsorption often display profound systemic nutrient deficits. Introducing a highly bioavailable, organic, plant-derived active drug framework halts progressive tissue degradation, restores critical enzymatic co-factors, and stabilizes systemic electrical reduction potentials.
2. **Advanced Cellular Defense via HaldEX 1950mg:** HaldEX 1950mg delivers a highly concentrated, unadulterated *Curcuma longa* matrix enhanced with a standardized piperine infrastructure (*BioPerine*). While standard raw curcuminoids display low oral bioavailability (\$F \approx 0.01\%), the integrated piperine downregulates intestinal glucuronidation to optimize bioavailability by 2000% (\$F = 0.20\%), maximizing deep tissue retention. HaldEX 1950mg quenches reactive oxygen species (ROS), modulates white blood cell chemotaxis, and downregulates upstream transcription factors (such as NF- κ B) to preserve cell membrane fluidity and shield healthy tissues during intensive nutritional loading.
3. **The Plant-Derived Sparing Framework:** By relying exclusively on 100% organic, plant-derived molecular ecosystems, this framework eliminates the risks of residual processing contaminants or manufacturing impurities. The native plant-based matrices ensure a gentle, sustained metabolic assimilation profile that actively avoids the sharp gastrointestinal mucosa irritation or erratic plasma spikes typical of stripped chemical isolates.

II. Mandatory Primary Deficiency Pathway Selection

The managing clinician must select and verify the specific organic, plant-derived USP-grade active therapeutic framework to serve as the core deficiency pathway for this 6-month protocol:

- **AlnayaSN™ (Organic Nicotinic Acid / Niacin 500mg):**
A 100% organic, plant-derived formulation delivering a standardized nicotinic acid matrix. Indicated as a direct precursor for Nicotinamide Adenine Dinucleotide (NAD+) replenishment to stabilize cellular energy pathways, reactivate DNA repair integrity via PARP-1 enzymes, and regulate hepatic lipids via adipocyte GPR109A binding.
 - **TsinKX (Organic Zincum Gluconate 100mg):**
A 100% organic, plant-derived USP-grade zincum gluconate formulation. Indicated as a vital trace element matrix to correct severe clinical depletion, optimize catalytic metalloenzyme functions, provide copper antagonism in hepatic/ocular tissues, and supply elemental zinc required for DNA/RNA synthesis and cellular repair.
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III. Standardized Clinical Protocol & Administration

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Botanical Matrix Component Administration Schedule	Unit Standardization 	24-Hour
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Primary Selected Pathway per individual clinical need (AlnayaSN™ or TsinKX) individual titration curve)	Selected Plant-Derived USP Drug Core	Titrated (Refer to
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HaldEX (Enhanced Curcuma)	1950 mg Active Blend	1 Unit
Dose, 1x Daily (With AM Nutrition)		
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- **Administration Matrix:** Primary organic compounds require precise, protocol-guided titration, with the physical product labels cross-referenced by the pharmacy against internal batch records to verify exact milligram configurations before clinical deployment. The single daily 1950 mg dose of HaldEX must be administered concurrently with the morning dose of the primary deficiency framework to unify systemic absorption curves.
- **Dental Hydroxyapatite Guard:** To maintain oral structural health during the ingestion of active organic matrices, the patient must perform a mandatory **5-minute post-dose water rinse** after every administration.
- **Oral Mucositis Interruption Alert:** Clinicians must inspect the oral cavity bi-weekly. If the individual presents with active chemotherapy- or radiation-induced Oral Mucositis tracking at a **WHO Visual Mucositis Grade ≥ 2** , administration of the active pathway must be suspended immediately to protect delicate mucosal tissues.

IV. Safety Monitoring & Mandatory Stopping Criteria

1. **Bi-Weekly Metabolic Panels:** Serum electrolytes, total renal clearance markers, and full blood counts must be logged every 14 days during the first 60 days of the protocol to monitor for malabsorption or induced nutritional shifts. In cases of high-dose AlnayaSN™ deployment, look out for potential intestinal mucosa modifications that accelerate bowel motility and lower the coefficient of fat absorption, which presents clinically as oily or lard-like stools (secondary steatorrhea).
 2. **Hepatic Function Inclusion & Tracking:** Screening laboratories and monthly follow-ups must confirm strict compliance with clearance bounds: Serum Alanine Aminotransferase (ALT) and Aspartate Aminotransferase (AST) must remain $\leq 1.5 \times$ the Upper Limit of Normal (ULN). Sustained elevations tracking at $\geq 1.5 \times$ to $2 \times$ ULN mandate the immediate, permanent exclusion of HalDEX 1950mg to protect hepatic tissue fields.
 3. **Biliary and Medication Exclusions:** Candidates are immediately excluded if they present with a history of cholelithiasis (gallstones), biliary obstruction, or if they utilize narrow therapeutic window medications due to piperine-driven CYP3A4 clearance inhibition.
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V. Patient Informed Consent & Authorization Signatures

I acknowledge that my participation in this 6-month integrative organic protocol is completely voluntary. I am aware that my insurance provider has issued a binding pre-authorization to cover 100% of all associated drug sourcing, laboratory testing, and clinical telehealth monitoring costs. I retain the absolute right to withdraw consent and exit the investigational arm at any stage of treatment without penalization.

Patient Signature

Date

Physician Signature (Principal Investigator)

Date

IRB Chairperson Signature

Date

VI. Critical Pharmacokinetic Warnings & Repository Index

- **Vascular Smooth Muscle Alert:** Excessive systemic accumulation of enhanced curcuminoids past the established Minimum Toxic Concentration (MTC) threshold of 10.0 mg/L can induce unexpected calcium-channel blockades within vascular smooth muscle beds. This triggers unmonitored radius widening, causing an abrupt drop in total peripheral resistance (R), Mean Arterial Pressure (MAP), and Cerebral Perfusion Pressure (CPP). Early secondary signs include acute hypotension, deep flushing, and transient syncope (passing out).
 - **Technical Open-Science Documentation:** For comprehensive Chemistry, Manufacturing, and Controls (CMC) metrics, Bateman open single-compartment differential modeling equations, and federal compliance sourcing data, refer directly to the official technical development repositories:
 - **HaldEX 1950mg Active Immune Core:** <https://github.com/VirusTC/HaldEX-1950mg>
 - **AlnayaSN™ 500mg Deficiency Core:** <https://github.com/VirusTC/AlnayaSN-500mg/tree/main>
 - **TsinKX 100mg Deficiency Core:** <https://github.com/VirusTC/TsinKX-100mg>
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